ED Throughput Scorecard: Findings Report

Emergency department wait times and walkouts at 4,658 US hospitals | October 26, 2026

Executive summary

At the typical US hospital, a patient sent home from the emergency department spends 148 minutes there (national median). That time depends heavily on hospital size and location. Across 4,658 hospitals, three findings matter most:

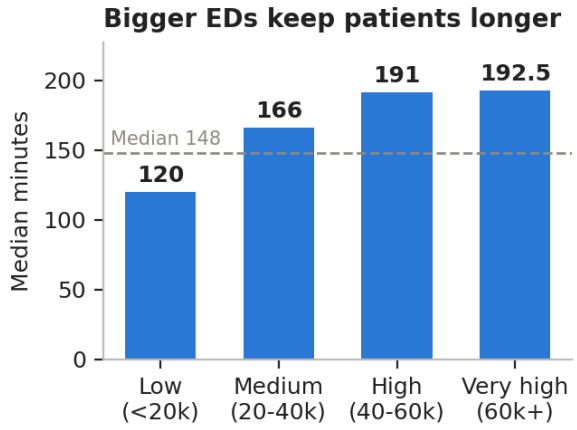
- **Bigger, busier EDs are slower.** Low-volume EDs have a median of 120 minutes; the busiest are near 192.
- **Slower EDs lose more patients.** Patients leaving before they are seen rise from 0.7% in the fastest quartile of hospitals to 3.0% in the slowest.
- **Mental health patients wait far longer.** At the same hospitals, they stay a median of 247 minutes versus 157 for everyone else, a gap of 83 minutes.

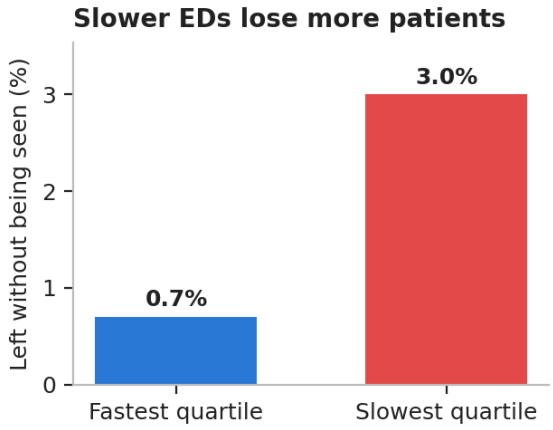
The Tableau dashboard lets anyone compare a hospital with what its size, type and location would predict, so a slow ED can be separated from one that is just big or rural.

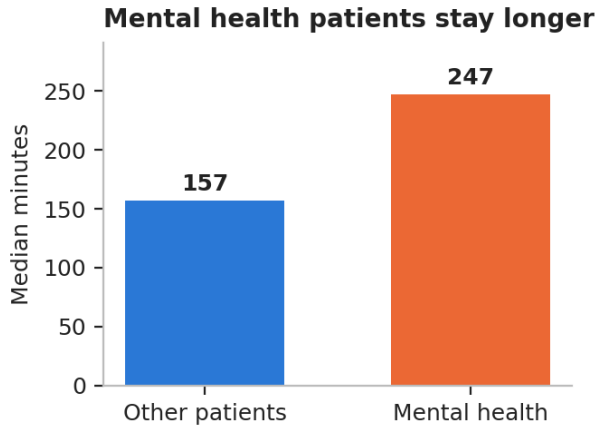
Key takeaways

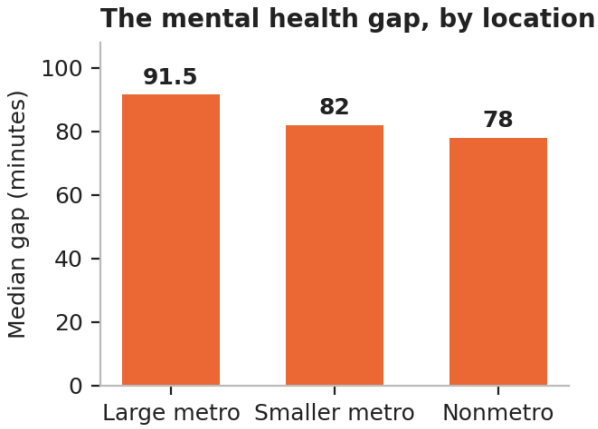
1. **Size drives wait time.** The typical ED stay is 120 minutes at low-volume hospitals, 166 at medium, 191 at high and 192.5 at very high volume. Critical access hospitals (small, rural) are quickest at 114.
2. **Long waits and walkouts go together.** Hospitals with longer stays also see more patients leave without being seen (rank correlation 0.56).
3. **Mental health patients wait about 90 minutes longer.** The typical gap is 83 minutes (247 vs. 157). At 92.5% of hospitals the mental health wait is longer, and at 22% those patients stay at least twice as long.
4. **Location matters.** Among states with at least 10 hospitals, the fastest medians are North Dakota (105), South Dakota (112) and Nebraska (114); the slowest are Maryland (241), Rhode Island (218.5), Massachusetts (211.5) and New York (204).
5. **A fair comparison is possible.** After allowing for size, hospital type, ownership, region and rurality, 912 hospitals are longer than expected, 2,077 are close to expected and 1,092 are shorter.

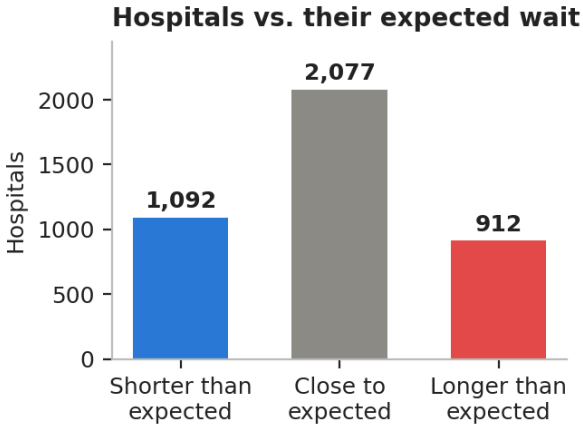
The findings in charts











Charts redrawn from the report's figures in the dashboard's color scheme (blue = shorter or single series, orange = mental health, gray = close to expected, red = longer than expected).

Numbers at a glance

Measure	Result
Typical ED stay, all hospitals (median)	148 min
Low-volume vs. very-high-volume EDs	120 vs. 192.5 min
Patients leaving unseen, fastest vs. slowest quartile	0.7% vs. 3.0%
Mental health patients vs. other patients	247 vs. 157 min
Hospitals where mental health patients wait longer	92.5%
Mental health gap: large metro / smaller metro / nonmetro	91.5 / 82 / 78 min

Limits to keep in mind

- **Discharged patients only.** The wait measure leaves out admitted patients who sit in the ED waiting for a bed. A new CMS measure (ECAT) will cover this: voluntary reporting in 2027, mandatory in 2028.
- **Sicker patients are not adjusted for.** A hospital that treats more complex cases may look slow for good reason.
- **Walkout rates are rounded.** CMS publishes them as whole-number percentages.
- **Association, not cause.** Long waits and walkouts rise together, but this data cannot show that one causes the other.
- **Territories are thin.** Only 8 territory hospitals have a wait score.

About the data

- **Sources:** CMS Timely and Effective Care and Hospital General Information (August 2026 release), plus USDA 2023 Rural-Urban Continuum Codes. All 4,658 hospitals matched to a rurality category.
- **Periods:** ED wait times cover Oct 2024 to Sep 2025; walkouts and ED volume cover calendar 2024.
- **Expected wait model:** a regression on volume, hospital type, ownership, region and rurality, tested on 4,081 hospitals. It explains about 48% of the variation between hospitals (cross-validated R-squared 0.482), so it is a fair benchmark, not a verdict on any one hospital.